SUMMIT GASTROENTEROLOGY & ENDOSCOPY CENTER

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COLONOSCOPY PROCEDURE REPORT

Patient Name:	Echevarría, Ester
Date of Birth:	06/20/1927
Age:	92 years
Sex:	Female
Medical Record Number:	MRN-847392
Date of Procedure:	September 17, 2019
Referring Physician:	Dr. Margaret L. Thornton, MD
Attending Physician:	Dr. Samuel R. Weinberg, MD, FACG

INDICATION FOR PROCEDURE

92-year-old female with history of recurrent rectal polyps presenting with rectal bleeding over the past two weeks. Patient has history of diabetes mellitus type II, atrial fibrillation on anticoagulation, Alzheimer's disease, chronic kidney disease, and multiple prior colonoscopies with polypectomies. Colonoscopy indicated for evaluation of bleeding and surveillance.

MEDICATIONS

Warfarin held 5 days prior to procedure (last dose 09/12/2019). INR confirmed 1.4 on day of procedure. Patient continued on Digoxin, Verapamil, Galantamine, and Simvastatin. Insulin dose adjusted per endocrinology.

PROCEDURE DETAILS

Sedation: Monitored anesthesia care provided by Dr. Patricia Nguyen, MD. Propofol 120 mg IV and Fentanyl 75 mcg IV administered. Patient monitored continuously with pulse oximetry, blood pressure, and cardiac telemetry given atrial fibrillation history.

Bowel Preparation: GoLYTELY split-dose preparation. Prep quality rated as good (Boston Bowel Preparation Scale: right colon 3, transverse 3, left colon 2). Some residual liquid stool in left colon, suctioned and cleared.

Scope: Olympus CF-H190L colonoscope

Extent of Examination: Cecum reached and identified by visualization of ileocecal valve and appendiceal orifice at 14 minutes. Total procedure time 38 minutes. Withdrawal time 22 minutes.

FINDINGS

Location	Description	Size (mm)	Intervention
Rectum (6 cm from anal verge)	Sessile, ulcerated mass with irregular surface and friable tissue. Spontaneous bleeding noted.	~25 mm	Multiple cold forceps biopsies (6 specimens) obtained. Lesion not amenable to endoscopic resection given size, morphology, and concern for invasive features.
Sigmoid colon	Pedunculated polyp, Paris classification Ip, benign appearance	8 mm	Snare polypectomy. Retrieved. Submitted as specimen B.
Descending colon	Sessile polyp, Paris IIa, smooth surface	6 mm	Cold snare polypectomy. Retrieved. Submitted as specimen C.
Transverse colon	Diminutive polyp	3 mm	Cold forceps biopsy. Submitted as specimen D.
Cecum	Diminutive polyp	2 mm	Cold forceps biopsy. Submitted as specimen E.

Additional Findings: Multiple diverticula noted in sigmoid and descending colon without inflammation. Mild internal hemorrhoids visualized on retroflexion in rectum.

ASSESSMENT

Rectal mass with features highly suspicious for malignancy including ulceration, friability, irregular borders, and spontaneous bleeding. Four additional polyps identified and removed from sigmoid, descending, transverse colon, and cecum.

PLAN

Patient tolerated procedure well without immediate complications. Transferred to recovery in stable condition. Specimens submitted to surgical pathology for histologic evaluation. Results pending and will be communicated to patient and referring physician.

Given high suspicion for colorectal malignancy, patient will require:

- Oncology consultation upon pathology confirmation
- Staging CT chest/abdomen/pelvis with IV contrast
- Colorectal surgery consultation
- CEA level
- Multidisciplinary tumor board review

Warfarin may be resumed 24 hours post-procedure if no bleeding. Bridge with low molecular weight heparin per cardiology recommendations given atrial fibrillation and elevated stroke risk.

Samuel R. Weinberg, MD, FACG

Board Certified Gastroenterology and Internal Medicine

Date: September 17, 2019

Electronically signed: 09/17/2019 14:22 EST

BOSTON PATHOLOGY ASSOCIATES

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CLIA #22D1098432

SURGICAL PATHOLOGY REPORT

Patient Name:	Echevarría, Ester
Date of Birth:	06/20/1927
Medical Record Number:	MRN-847392
Pathology Accession:	S19-28477
Date Collected:	09/17/2019
Date Received:	09/17/2019
Date Reported:	09/20/2019
Physician:	Dr. Samuel R. Weinberg, MD
Pathologist:	Dr. Katherine M. O'Sullivan, MD, PhD

CLINICAL HISTORY

92-year-old female with rectal bleeding. Known history of recurrent colonic polyps. Multiple specimens submitted from colonoscopy.

GROSS DESCRIPTION

Specimen A (Rectal mass biopsies): Received fresh in formalin, labeled "rectal mass biopsies," are six fragments of gray-tan tissue measuring 2-4 mm each. Entirely submitted in one cassette (A1).

Specimen B (Sigmoid polyp): Received fresh in formalin, labeled "sigmoid polyp," is a single polypoid fragment of tan-pink tissue measuring 8 x 6 x 4 mm. Bisected and entirely submitted (B1).

Specimen C (Descending colon polyp): Received fresh in formalin, labeled "descending colon polyp," is a single polypoid fragment measuring 6 x 5 x 3 mm. Entirely submitted (C1).

Specimen D (Transverse colon polyp): Received fresh in formalin, labeled "transverse colon polyp," is a single fragment measuring 3 x 2 x 2 mm. Entirely submitted (D1).

Specimen E (Cecal polyp): Received fresh in formalin, labeled "cecal polyp," is a single diminutive fragment measuring 2 x 2 x 1 mm. Entirely submitted (E1).

MICROSCOPIC DESCRIPTION

Specimen A (Rectal mass biopsies): Sections demonstrate colonic mucosa with invasive adenocarcinoma. The carcinoma is composed of moderately differentiated glands with irregular architecture infiltrating through the muscularis mucosae into the submucosa. Malignant glands exhibit moderate nuclear pleomorphism, increased nuclear-to-cytoplasmic ratio, hyperchromasia, and prominent nucleoli. Mitotic figures are readily identified. Areas of necrosis are present. No lymphovascular invasion is identified in the limited biopsy material. Adjacent mucosa shows fragments of tubulovillous adenoma with high-grade dysplasia.

Specimen B (Sigmoid polyp): Sections reveal a tubular adenoma with low-grade dysplasia. The polyp demonstrates elongated, hyperchromatic nuclei with preserved polarity and minimal architectural complexity. No high-grade dysplasia or invasive carcinoma identified. Excision appears complete.

Specimen C (Descending colon polyp): Sections show a tubular adenoma with low-grade dysplasia. Cytologic and architectural features consistent with adenomatous change without high-grade dysplasia or malignancy.

Specimen D (Transverse colon polyp): Sections demonstrate a hyperplastic polyp with characteristic serrated architecture of crypts, mature goblet cells, and lack of cytologic atypia. No dysplasia identified.

Specimen E (Cecal polyp): Sections show a hyperplastic polyp with serrated crypt architecture. No dysplasia or malignancy present.

DIAGNOSIS

A. RECTUM, BIOPSIES:

- INVASIVE MODERATELY DIFFERENTIATED ADENOCARCINOMA
- Arising in association with tubulovillous adenoma with high-grade dysplasia
- Depth of invasion: At least submucosal (limited by biopsy sampling)
- Lymphovascular invasion: Not identified

B. SIGMOID COLON, POLYPECTOMY:

- TUBULAR ADENOMA WITH LOW-GRADE DYSPLASIA
- Margins: Negative (cauterized margin present)

C. DESCENDING COLON, POLYPECTOMY:

- TUBULAR ADENOMA WITH LOW-GRADE DYSPLASIA
- Margins: Negative

D. TRANSVERSE COLON, BIOPSY:

- HYPERPLASTIC POLYP

E. CECUM, BIOPSY:

- HYPERPLASTIC POLYP

COMMENT

The rectal biopsies demonstrate invasive moderately differentiated adenocarcinoma. Given the biopsy nature of the specimen, full extent of tumor invasion cannot be assessed. Clinical correlation with endoscopic findings and imaging studies is recommended. Definitive surgical resection will be required for complete staging, including assessment of depth of invasion (T stage), lymph node status (N stage), and margin status.

The patient has synchronous adenomatous polyps in the sigmoid and descending colon with low-grade dysplasia, which have been completely excised endoscopically. Two diminutive hyperplastic polyps were also identified.

Recommend:

- Oncology consultation for treatment planning
- Staging imaging (CT chest/abdomen/pelvis)
- Colorectal surgical consultation for definitive resection
- Consideration of multidisciplinary tumor board discussion
- Molecular testing (microsatellite instability, KRAS, NRAS, BRAF) on resection specimen for prognostic and therapeutic stratification

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Katherine M. O'Sullivan, MD, PhD

Board Certified Anatomic and Clinical Pathology

Subspecialty: Gastrointestinal Pathology

Date: September 20, 2019

Electronically signed: 09/20/2019 16:45 EST

Distribution: Dr. Samuel R. Weinberg (Gastroenterology), Dr. Margaret L. Thornton (Referring Physician), Medical Record

Note: This report has been reviewed and discussed with the clinical team. Oncology referral placed. Patient and family notified of findings on 09/20/2019.